Administrative Changes to AFMD 35, *Air Force Medical Readiness Agency*

OPR: AFMED/CSS

References throughout to “Air Force Medical Readiness Agency (AFMRA)” are hereby changed to “Air Force Medical (AFMED)”.

5 September 2024.

BY ORDER OF THE SECRETARY OF THE AIR FORCE

AIR FORCE MISSION DIRECTIVE 35

11 MAY 2022

AIR FORCE MEDICAL READINESS AGENCY

COMPLIANCE WITH THIS PUBLICATION IS MANDATORY

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RELEASABILITY: There are no releasability restrictions on this publication.

OPR: AFMRA/CSS

Certified by: AFMRA/CC
(Brigadier General Thomas W. Harrell)

Supersedes: AFMD 35, 19 October 2016 and AFMD 36, 03 October 2016

Pages: 4

This directive states the mission, defines the command structure, and establishes the responsibilities of the Air Force Medical Readiness Agency (AFMRA) as a United States Air Force Field Operating Agency (FOA) directly subordinate to the Surgeon General of the Air Force. Refer recommended changes and questions about this publication to the Office of Primary Responsibility (OPR) using the AF Form 847, *Recommendation for Change of Publication*; route AF Forms 847 from the field through appropriate functional chain of command. Ensure all records generated as a result of processes prescribed in this publication adhere to Air Force Instruction 33-322, *Records Management and Information Governance Program*, and are disposed in accordance with the Air Force Records Disposition Schedule, which is located in the Air Force Records Information Management System.

1. Mission. AFMRA supports United States Air Force (USAF), United States Space Force (USSF) and joint medical missions through the execution of Department of the Air Force medical programs and a framework of high reliability processes that enhance medical readiness and ensure a ready medical force. It supports the health and performance of Service members through medical training, preventive medicine, enhanced performance and clinical currency for readiness activities in Military Treatment Facilities (MTFs) on Air Force and Space Force installations and other Department of the Air Force (DAF) medical units. AFMRA is the Surgeon General's readiness agency and primary focal point for execution and standardization of plans, practices, procedures and programs in force development and management, medical operations, logistics, planning and programming, and analytics for the Air Force Medical Service (AFMS). AFMRA collects, validates, analyzes and presents data to users at every level of the AFMS and coordinates readiness performance and process improvement activities in support of the AFMS strategy. In order to better execute the duties of the Department of the Air Force and the Air Force Surgeon General, as described in Section 712 of the National Defense Authorization Act of 2019, AFMRA will oversee:

1.1. Organizing, training and equipping uniformed medical and dental personnel.

1.2. Assigning of these personnel to military treatment facilities and other Sister-service or federal facilities, or non-MTF functions.

1.3. Ensuring the readiness for operational deployments of these medics.

1.4. Providing logistical support for operational deployments, maintenance and sustainment of Defense Support for Civil Authority and Homestation Medical Response assets.

1.5. Increasing deployability by focusing on ensuring a "medically ready" operational force.

2. Command.

2.1. The Air Force Medical Readiness Agency (AFMRA) is a field operating agency (FOA) reporting to AF/SG.

2.2. **The AFMRA Commander (AFMRA/CC):**

2.2.1. Commands assigned forces.

2.2.2. Is responsible to the Air Force Surgeon General (AF/SG) for administration, training and readiness of assigned forces.

2.2.3. Serves as AF/SG's lead on medical readiness issues.

2.2.4. Provides support to Department of Defense (DoD), Headquarters Air Force, major commands (MAJCOMs), field commands (FLDCOMs), director reporting units (DRUs) and FOAs, MTFs on Air Force and Space Force installations, other federal agencies and external civilian medical and research organizations on behalf of AFMS interests.

2.2.5. In coordination with the Medical Operations Directorate (AF/SG3/4) and the Manpower, Personnel and Operational Resourcing Directorate (AF/SG1/8), supports and oversees execution of AF/SG guidance and programs.

3. Responsibilities.

3.1. The AFMRA/CC:

3.1.1. Supports AF/SG policy development and execution while supporting the MAJCOMs and base-level unit mission execution.

3.1.2. Supports the AF/SG and MAJCOMs in the execution of the operational medical mission, provides program support for a medically-ready force and medical force readiness in support of the Global Force Management (GFM) construct, Joint Staff taskings, and related Request for Forces from Combatant Commanders (CCDR).

3.1.3. Assists in the development and execution of operational policy, plans, procedures and decisions on providing a fit and healthy force while supporting the AFMS operational readiness mission.

3.1.4. Supports AF full-spectrum medical readiness capability ensuring the right number of medics are properly led, trained, and equipped to accomplish the AF mission in support of the joint force in both contested and uncontested environments.

3.1.5. Measures readiness capability through critical skills availability, training resource availability, volume, diversity, and acuity of care in MTFs, Comprehensive Medical Readiness Program (CMRP), and operational tempo.

3.1.6. Supports capability development through requirements management, medical test and evaluation and integration with the Air Force Materiel Command (AFMC) and Defense Health Agency (DHA) Directorates for Acquisition and Research & Development.

3.1.7. Reports to the AF/SG and liaises with DHA and the military health system markets to provide direct service support to the MTFs in the execution of AFMS and DHA policies and programs, in coordination with, and while aligning efforts with MTF directors, MAJCOM surgeons, the military health system, other service department branches and key mission partners, the Military Health System, other Service Department branches and key mission partners.

3.1.8. Coordinates with the DHA to ensure clinical activities meet the necessary requirements of Congressionally mandated Department of Defense medical readiness programs.

3.1.9. Directs Clinical Quality Management Support such as clinical quality, patient safety, healthcare risk management, credentials and privileging, infection prevention, and compliance programs needed to support operational forces and missions

3.1.10. Manages the allocation and utilization of non-Defense Health Program (DHP) manpower and funds for DAF MTFs in accordance with programming and budgeting by the Manpower, Personnel and Resources Directorate (AF/SG1/8) and coordinates such allocations and utilizations with the DHA.

3.1.11. Develops recommendations for the AF/SG in coordination with other Directorates regarding optimal apportionment of resources to clinical processes to meet strategic priorities pertaining to readiness, provision of quality clinical care, and force development

3.1.12. Analyzes healthcare processes in the interest of high quality and safe care, efficient use of resources and clinical currency for readiness of uniformed medical personnel. Leads proposal and execution of AFMS health care process improvement initiatives as approved by the AF/SG.

3.1.13. Serves as the AFMS Services Designated Official (SDO) on behalf of the AF/SG and oversees and supports efficient, effective AFMS contracting activities.

3.1.14. Maintains the DAF Component Office of Human Research Protections (COHRP) to foster and oversee the compliance and ethics of all research, tests, and studies that involve human subjects across the DAF.

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Surgeon General